

Botulinum toxin for muscular TMD

For significant jaw-muscle symptoms, temporary reduction of muscle activity with botulinum toxin may be considered. A clear diagnosis, realistic goals and comparison with conservative options are essential.

TMD and bruxism are different

Temporomandibular disorders, or TMD, involve the jaw muscles, jaw joints or their interaction. Bruxism means repeated clenching, grinding or bracing of the jaw while awake or asleep. The conditions may occur together but are not interchangeable.

Muscle pain, tension and discomfort with chewing may be relevant. Dental causes, inflammatory or structural joint problems and other pain conditions need to be distinguished. Clicking alone is not a reason for an injection.

What treatment may achieve

Botulinum toxin type A temporarily reduces nerve signalling to the treated muscle. Muscle force may decrease, which can relieve selected overload-related symptoms. The masseter and, when appropriate, the temporalis muscle may be considered.

A possible goal

Less muscle pain or overload and better everyday function. We agree in advance how improvement would be assessed for you.

An important limitation

Clenching or grinding may continue just as often. Joint damage and other causes are not corrected. The effect is temporary.

When we consider this option

Particularly when distress is substantial, examination confirms a muscular component and suitable conservative measures have not provided sufficient relief. The 2026 German bruxism guideline allows consideration in adults, with low certainty of evidence. Other guidelines are more cautious about its use for chronic TMD pain overall.

Off-label use: Treatment of TMD or bruxism is outside the approved indications for this medicine. Expected benefit, uncertainty, risks and alternatives are therefore discussed explicitly. Improvement cannot be guaranteed.

Alternatives and accompanying treatment

Basic care includes understanding the symptoms, self-monitoring and appropriate jaw use. Depending on the findings, physiotherapy exercises, manual therapy and relaxation or behavioural approaches may help. A suitable splint can protect the teeth; it is not a solution for every cause of pain.

Sleep, stress, medication and associated conditions are considered. Persistent or complex pain may benefit from multidisciplinary care. Conservative measures can be continued or adjusted without injections.

Preparation and examination

- Discuss symptoms, their course and previous treatment; bring your splint and available records if relevant.
- List all medication and previous botulinum toxin treatments, including dates and product names. Some antibiotics and medicines affecting muscles may alter the effect.
- Tell us about allergies, muscle or nerve disease, swallowing or breathing problems, pregnancy or breastfeeding. Do not stop blood thinners yourself.
- No injection is given into an infected area or when treatment is unsuitable. Elective treatment is postponed during pregnancy and breastfeeding.

What happens during treatment

Following examination of the muscles and jaw function, we select the muscles, sides and extent of treatment. The skin is disinfected and small injections are given with fine needles. Dose and distribution are individual and product-specific. You remain awake.

Anaesthesia: Needle pricks may briefly hurt. Sedation or general anaesthesia is normally unnecessary. Cooling or suitable local measures can be discussed if needed. Complete absence of discomfort is not promised.

Possible risks

- Pain, redness, swelling, bruising, infection, headache or allergy.
- Temporary chewing weakness, a changed smile, asymmetry or dry mouth from effects on nearby structures.
- Reduced muscle bulk and altered facial contours. Long-term muscle and bone changes after repeated treatment remain incompletely understood.
- Rarely, weakness beyond the intended area and speech, swallowing or breathing problems. Severe allergic reactions are rare but possible.

Effect and follow-up

The effect is delayed, usually developing over the first days to weeks. At a review, often after 2–4 weeks, we assess pain, chewing strength, function and adverse effects. The effect fades over several months; duration and benefit vary.

Repeat treatment is reconsidered according to progress and product-specific minimum intervals. Frequent additional injections do not replace reassessment. If benefit is insufficient, the plan is reviewed.

After treatment

Do not firmly rub or massage the injection sites. Avoid strenuous exercise, strong heat and alcohol on the treatment day. These precautions also help limit local reactions. Agreed exercises and an appropriate splint remain part of your plan.

Contact us for marked chewing weakness, increasing swelling or pain, or noticeable asymmetry. Do not drive if vision or other relevant functions are affected. Use pain relief only if needed according to personal advice; do not change medication yourself.

New speech, swallowing or breathing problems or marked general muscle weakness require immediate medical assessment.

Practice **06151 786540**. Following treatment, outside practice hours: **+49 151 10437450**. If unavailable: emergency dental service **01805 60 70 11**.

For a life-threatening emergency, breathing difficulty, severe swallowing difficulty or rapidly increasing swelling of the floor of the mouth, neck or tongue: **112**.

Costs and private payment

German statutory health insurance does not usually cover this treatment. Costs are **agreed individually as a privately paid service** before treatment. The extent of treatment, review and possible repeat sessions are discussed; reimbursement is not guaranteed.

Frequently asked questions

Will grinding disappear completely?

This should not be expected. Treatment can reduce muscle force without eliminating the underlying behaviour. Tooth protection and other measures remain important.

Can the effect be reversed immediately?

No simple immediate reversal is available. Adverse effects are assessed and treated if necessary; the muscle effect usually has to wear off.

Consultation checklist

Diagnosis and goal understood? Conservative options reviewed? Off-label use and risks discussed? Costs, follow-up and contact details clear?

Clinical background

DGFD/DGZMK: S3 bruxism guideline 2026, recommendation 24 (German); BMJ: Chronic TMD pain guideline, 2023; Botulinum toxin type A: product information (German). Accessed 13 September 2026. Your findings and the selected product information determine its individual use.